NCCP 00605 · VERSION 2

Talazoparib · breast monotherapy

Talazoparib is taken once daily continuously until disease progression or unacceptable toxicity. The usual starting dose is 1 mg; kidney function, interactions and side effects can require a different prescription.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Anaemia, low white cells and platelets

☐ Rare serious bone-marrow disorders including MDS/AML

☐ Fatigue, nausea and vomiting

☐ Harm to an unborn baby

Discuss the exact setting, expected benefit, alternatives, medicine-specific risks, monitoring, contraception and urgent contact plan.

Individual benefit, alternatives and concerns

Agreed checks and next review

Check FBC, kidney and liver function before starting and every four weeks. More frequent blood review may be needed for low counts; prolonged abnormalities require specialist assessment. Confirm the germline BRCA result and previous treatment. Review pregnancy status and contraception, kidney function and interacting medicines, particularly P-glycoprotein inhibitors.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.19 · Sources checked 2026-10-25
Source reconciled; independent clinical, pharmacy and patient/PPI review pending